

Nixara

A recommendation tool. Nixara suggests the decision; you make it.

Every figure is worth checking before you act on it.

NIK·SA·RA /NIK'SCI:RƏ/ · from *nix* (clarity, light) + *ara* (direction) — illuminating the path forward in your data

Upload CSV / Excel · Connect Tableau or Power BI · Get executive-grade AI reports

- Dashboard
- Memory
- Inbox
- Outcomes
- FAQ

☯ Good afternoon.

Still the same **1 decision** from earlier today — they haven't gone anywhere. →

- Upload CSV / Excel
- Tableau
- Power BI

Drop a CSV or Excel file here, or click to browse
.csv, .xlsx, .xls

HEALTHCARE_DATASET.XLSX · 55500 ROWS

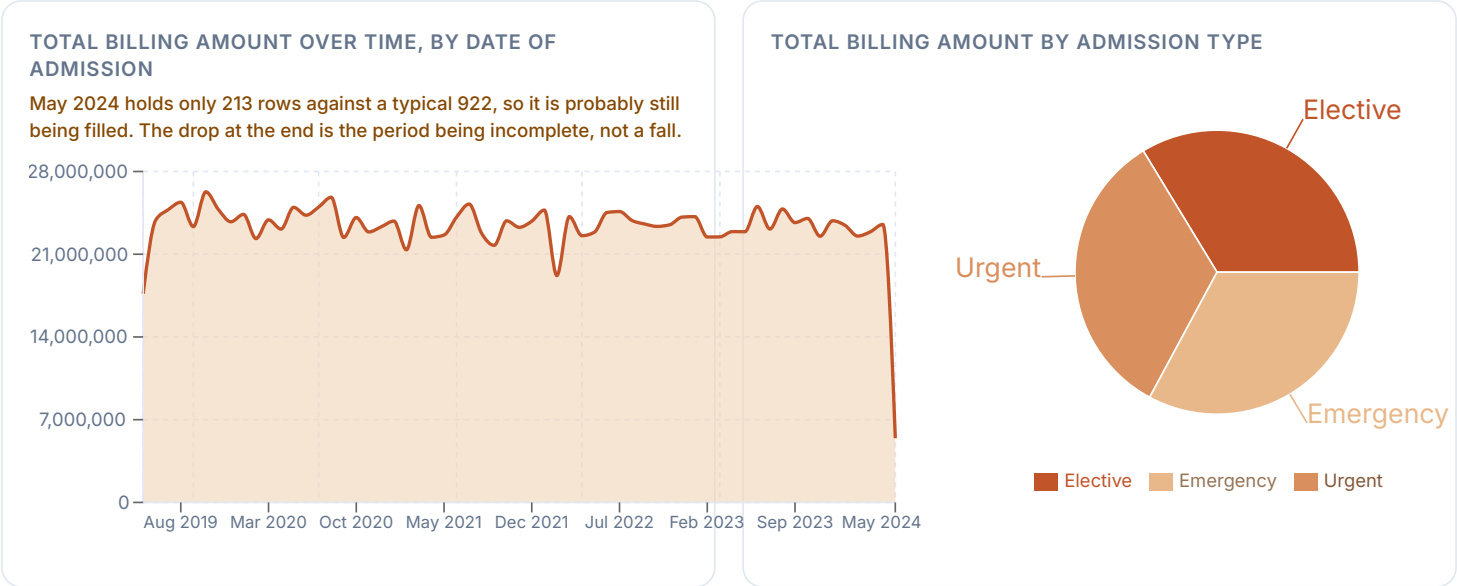
ROWS	COLUMNS	NUMERIC FIELDS	QUALITY SCORE
55,500	15	3	91

Good enough for decision support

WHY IT'S NOT 100

- ⚠ 534 rows are exact duplicates of another row (1.0% of the file), which inflates every total they appear in
- ⚠ Billing Amount contains 108 negative values, which net against the positives in every total unless they are meant to

Name	Age	Gender	Blood Type	Medical Condition	Date of Admission	Doctor	Hospital
Bobby Jackson	30	Male	B-	Cancer	Jan 31, 2024	Matthew Smith	Sons and
Leslie Terry	62	Male	A+	Obesity	Aug 20, 2019	Samantha Davies	Kim Inc
Danny Smith	76	Female	A-	Obesity	Sep 22, 2022	Tiffany Mitchell	Cook PL
Andrew Watts	28	Female	O+	Diabetes	Nov 18, 2020	Kevin Wells	Hernand
Adrienne Bell	43	Female	AB+	Cancer	Sep 19, 2022	Kathleen Hanna	White-W
Emily Johnson	36	Male	A+	Asthma	Dec 20, 2023	Taylor Newton	Nunez-H
Edward Edwards	21	Female	AB-	Diabetes	Nov 3, 2020	Kelly Olson	Group M
Christina Martinez	20	Female	A+	Cancer	Dec 28, 2021	Suzanne Thomas	Powell R
Jasmine Aguilar	82	Male	AB+	Asthma	Jul 1, 2020	Daniel Ferguson	Sons Ric
Christopher Berg	58	Female	AB-	Cancer	May 23, 2021	Heather Day	Padilla-V



Name any of these in your question to chart it: **Gender** (2) · **Blood Type** (8) · **Medical Condition** (6) · **Insurance Provider** (5) · **Admission Type** (3) · **Medication** (5) **+1 more**

REPORT SETUP

Common decisions (optional starting point)

Pricing / discounting

Cost reduction

Expansion / hiring

Customer retention

Operational efficiency

Risk / compliance

Decision Context

We need to cut billing exposure by 10% next year. Which condition, insurer or admission type should we target first, and what would that save?

Role

Time Horizon

COO

This quarter

OpenAI API Key (optional)

.....

Not stored — cleared when you close or refresh this tab.

✓ Key active

Generate Reports

AI REPORTS

- Executive Summary
- Operational Detail
- Risk Report

Reading this report: 🔍 source traced to your data 🧮 calculated computed by Nixara from your data
unverified could not be matched, may still be right

3 figures in this version could not be confirmed against your data.

0.9% Quick Win 1

\$53,925.00 Quick Win 2

10% Quick Win 3

Click a figure to jump to it. Check these before acting on them.

PERFORMANCE BREAKDOWN

The billing amounts across the admission types are nearly evenly distributed: Elective at \$477,609,530.85, Urgent at \$474,010,562.92, and Emergency at \$465,811,948.18. Each category accounts for approximately one-third of the total billing amount. The spread by medical condition is similarly uniform: Diabetes, Obesity, Arthritis, Hypertension, Asthma, and Cancer are all close in total billing, with Diabetes leading at \$238,539,725.40. Providers like Cigna account for 20.3% of the billing exposure, but this does not suggest a concentration risk.

EFFICIENCY GAPS

Based on the available data, no single condition, insurer, or admission type shows a significant disparity in billing amounts that would justify targeted savings. For instance, while Diabetes has the highest total billing, Obesity has a higher per-row average amount. Similarly, while Cigna leads, its share of total billing is not substantially above an even split with its peers to target as an efficiency gap.

PROCESS RECOMMENDATIONS

THIS WEEK

 Data Quality Team

Verify and resolve duplicate rows, which currently account for **\$13,363,704.00** in billing.

 Finance Team

Investigate the negative billing amounts totaling - **\$53,925.00** to determine if they represent errors or refunds.

THIS QUARTER

 Operations Team

Implement a comprehensive review process for billing procedures to address systemic inefficiencies and enhance data integrity.

QUICK WINS

0.9% unverified

Addressing the 534 duplicate rows immediately, which constitute **0.9%** of the total billing amount, can streamline operations without requiring additional resources, given their inclusion in the original billing data total of **\$1,417,432,041.95**.

\$53,925.00 unverified

Correcting the negative billing amounts, totaling - **\$53,925.00**, can improve the accuracy of financial reporting without any need for budget adjustments.

10% unverified

The target of cutting billing exposure by **10%**, equivalent to **\$141,743,204.00**, cannot be met by singularly focusing on any particular group, given the absence of significant variability in billing distribution across the current dimensions. Instead, this goal necessitates broad systemic improvements and process efficiencies.

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 Record Your Decision

What did you decide to do with these recommendations?

Which recommendation does this apply to? *

- ☐ All recommendations
- ☐ 1. [This week] Data Quality Team: Verify and resolve duplicate rows, which currently account for \$13,363,704.00 in billing.
- ☐ 2. [This week] Finance Team: Investigate the negative billing amounts totaling -\$53,925.00 to determine if they represent errors or refunds.
- ☐ 3. [This quarter] Operations Team: Implement a comprehensive review process for billing procedures to address systemic inefficiencies and enhance data integrity.
- ☐ Quick Win 1: Addressing the 534 duplicate rows immediately, which constitute 0.9% of the total billing amount, can streamline operations without requiring additional resources, given their inclusion in the original billing data total of \$1,417,432,041.95.

☐ Quick Win 2: Correcting the negative billing amounts, totaling -\$53,925.00, can improve the accuracy of financial reporting without any need for budget adjustments.

Who is responsible for this action? (optional)

e.g. Sarah Chen, CFO team, Operations dept...

Notes (optional)

Revisit end of Q3, pending budget approval...

Due by (optional — only applies if you approve)

10 / 10 / 2026

☒ Approve

☒ Reject

☒ Postpone